

SCAI First, Washington DC - Case Presentation

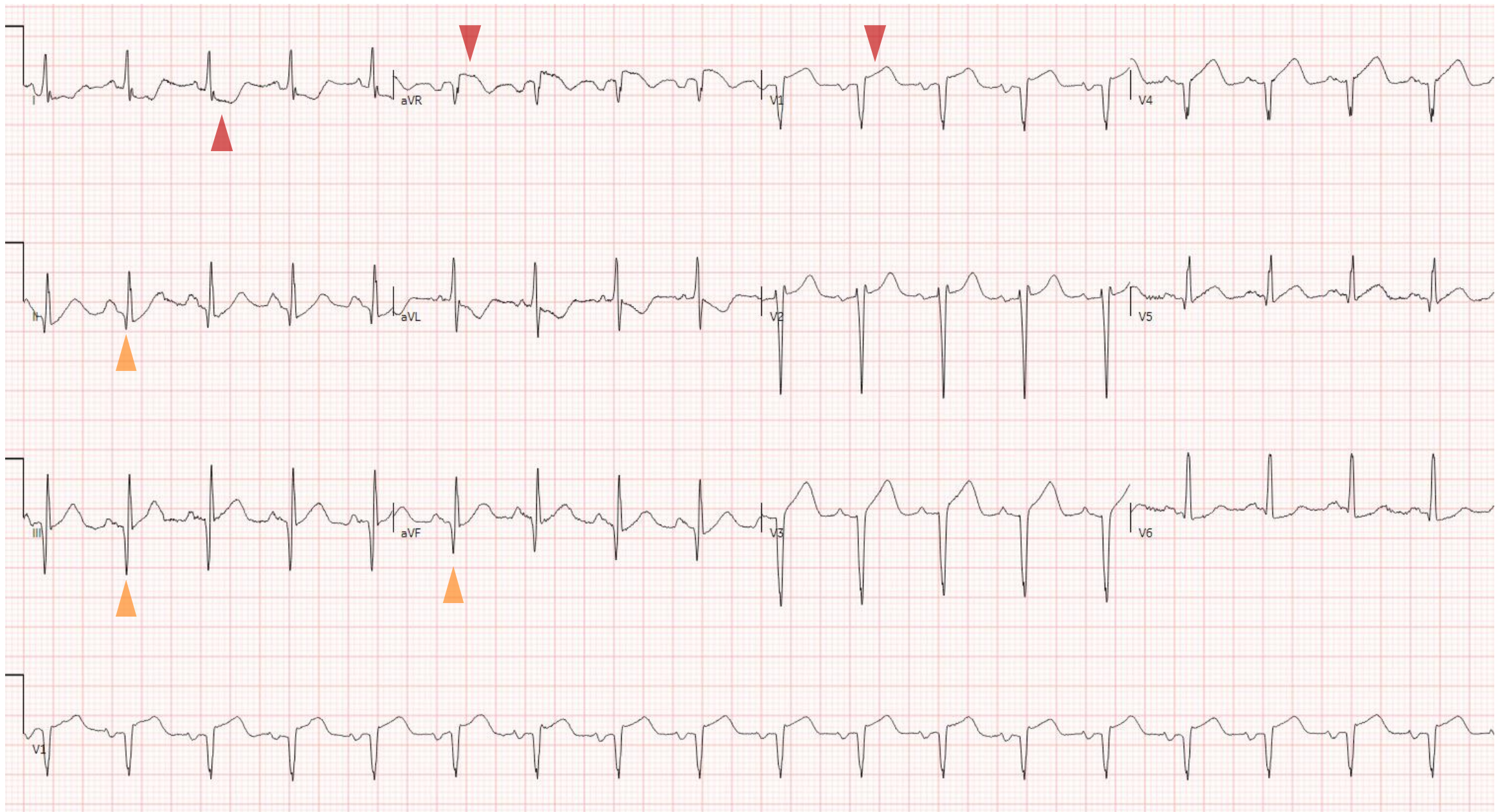
Better late than never: Late-presenting ACS with AMI-CS successfully treated with multivessel PCI

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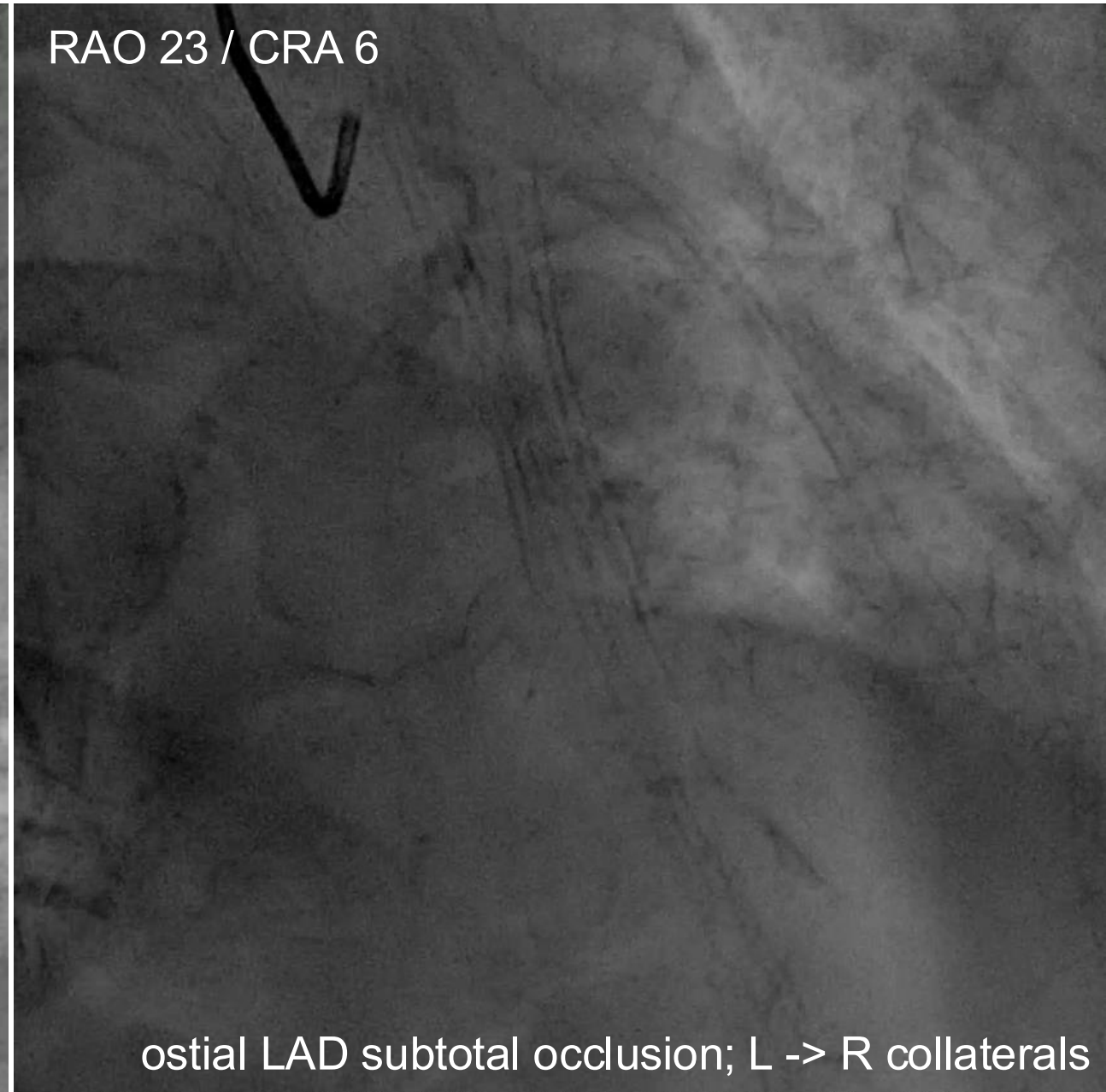
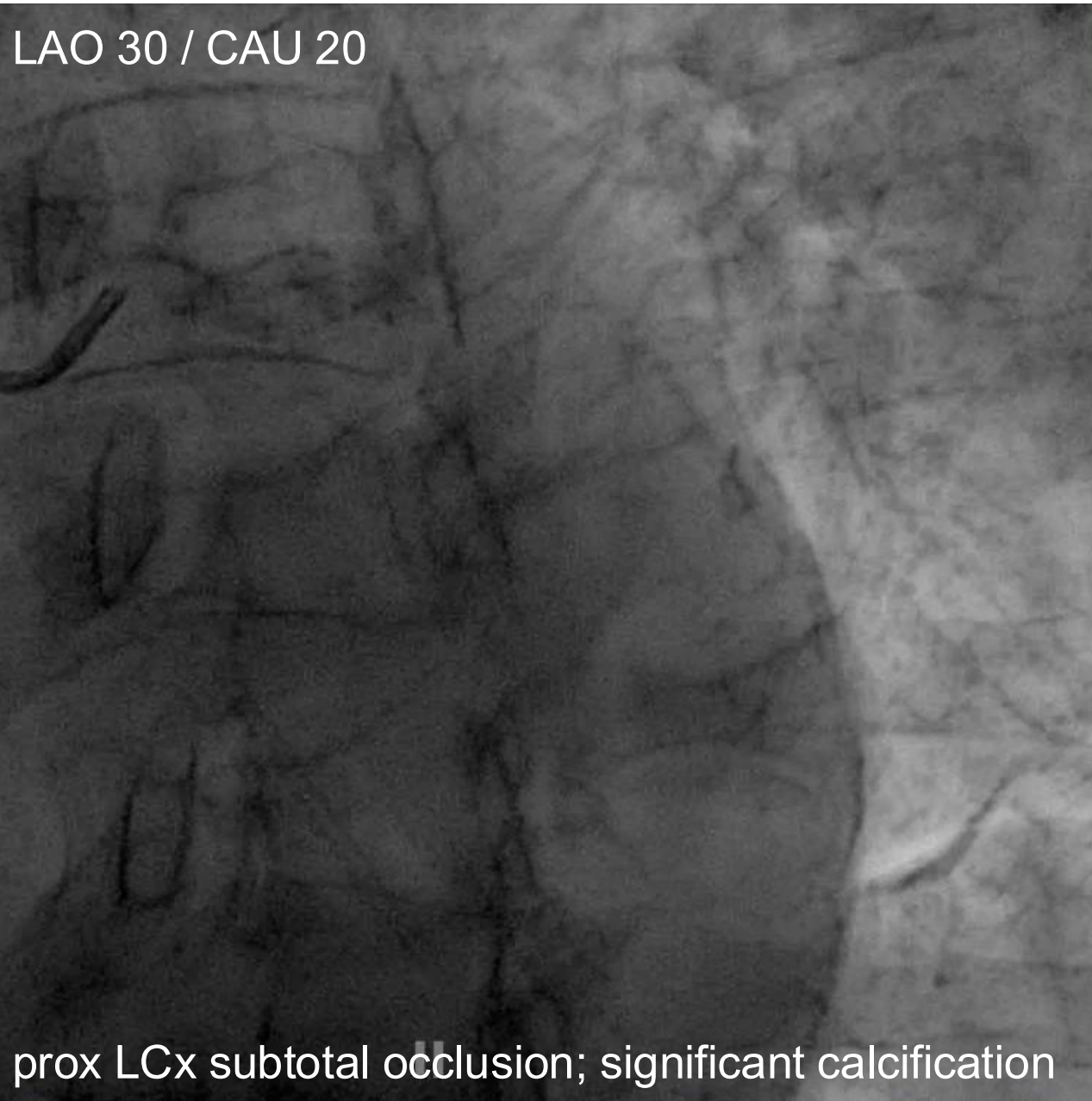


- 77-year-old female with only known PMH of HTN
- Presented through ED >14 hrs after onset of substernal chest pain the prior afternoon
 - Radiating to epigastrium and upper back; associated with dyspnea, diaphoresis, and nausea
- Never smoker; FHx father with MI in 70s
- Previously active able to tolerate swimming multiple days per week

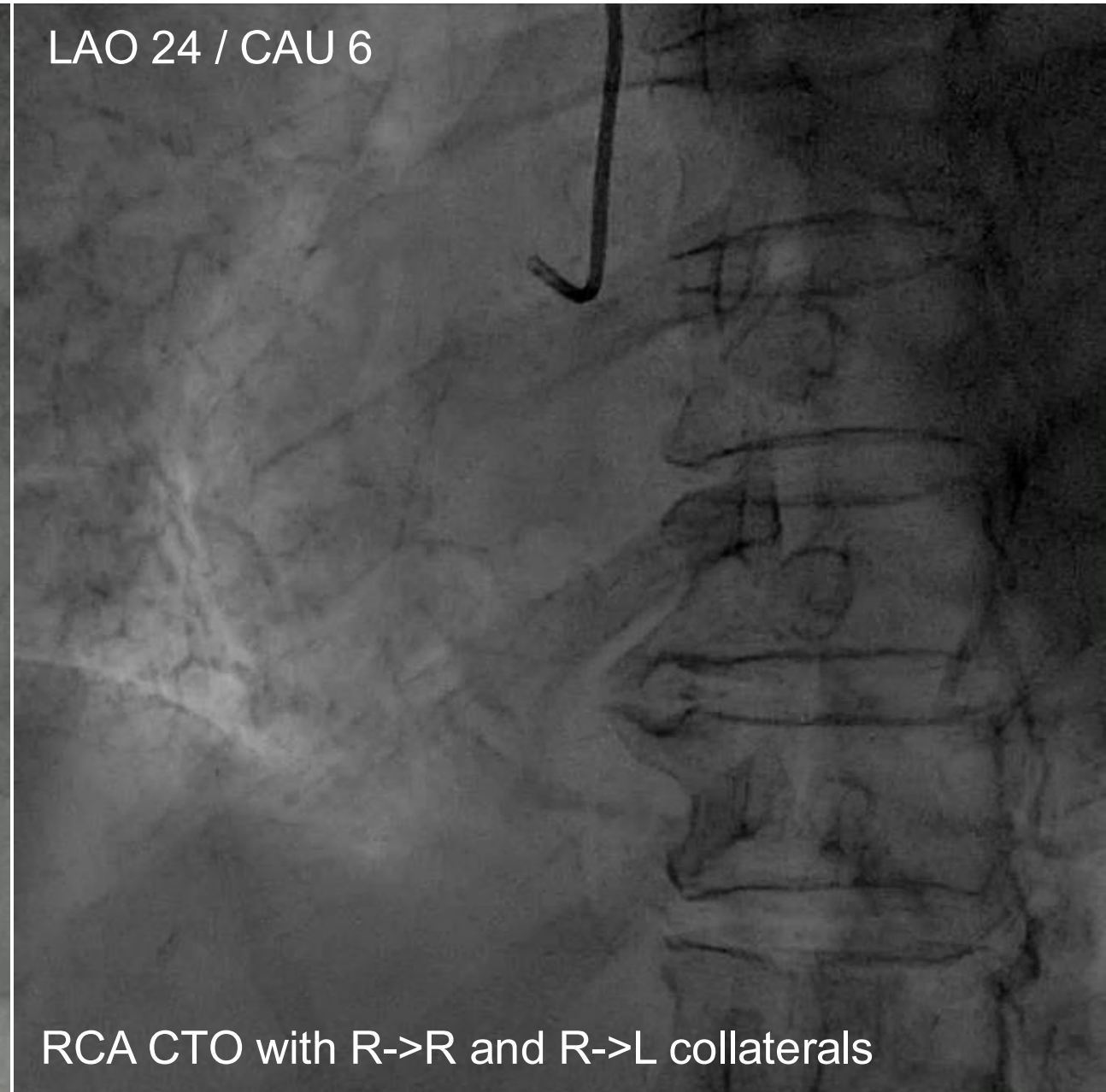
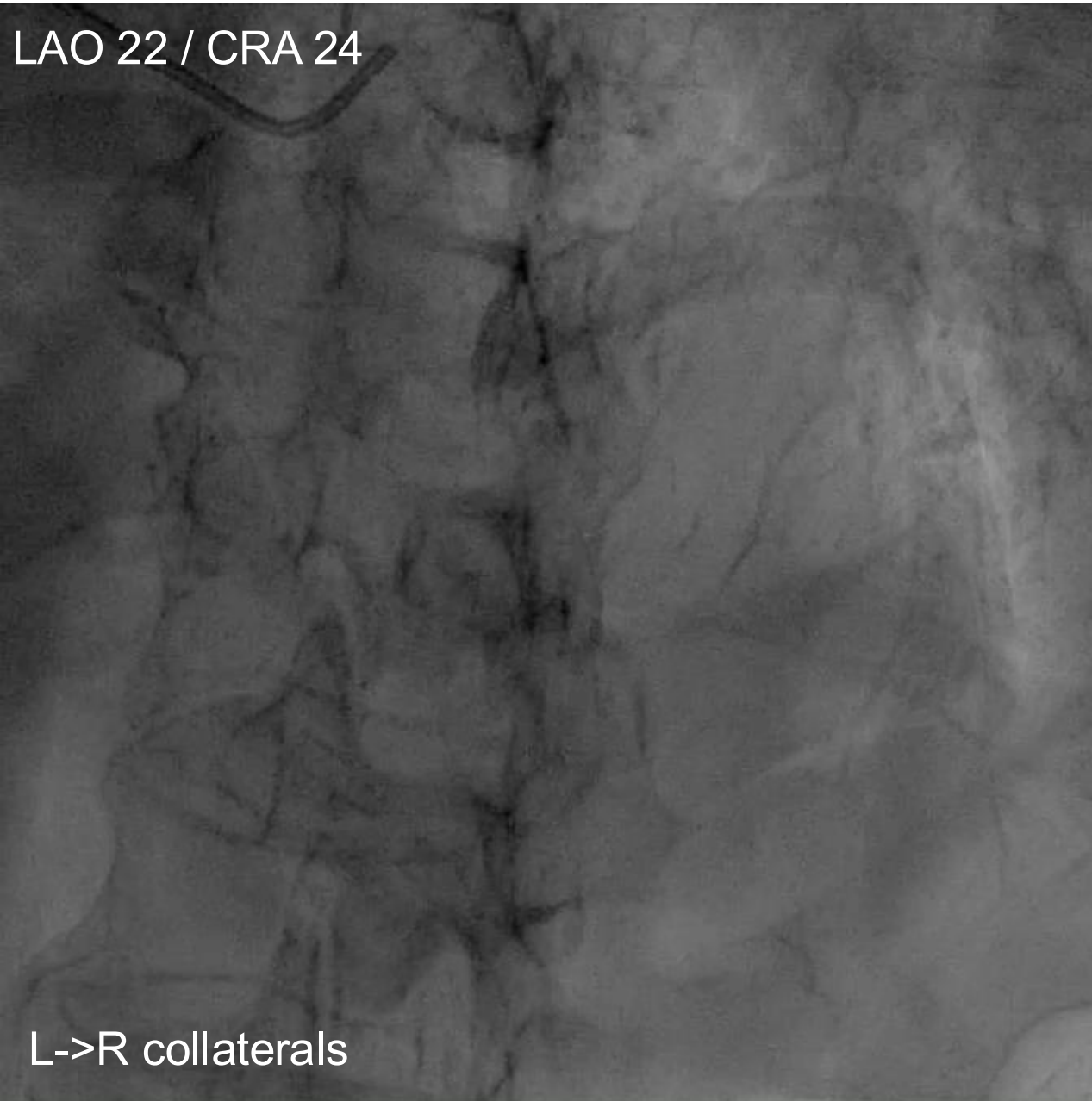
Presenting ECG



Emergent Coronary Angiography

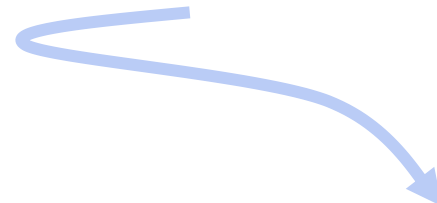


Emergent Coronary Angiography

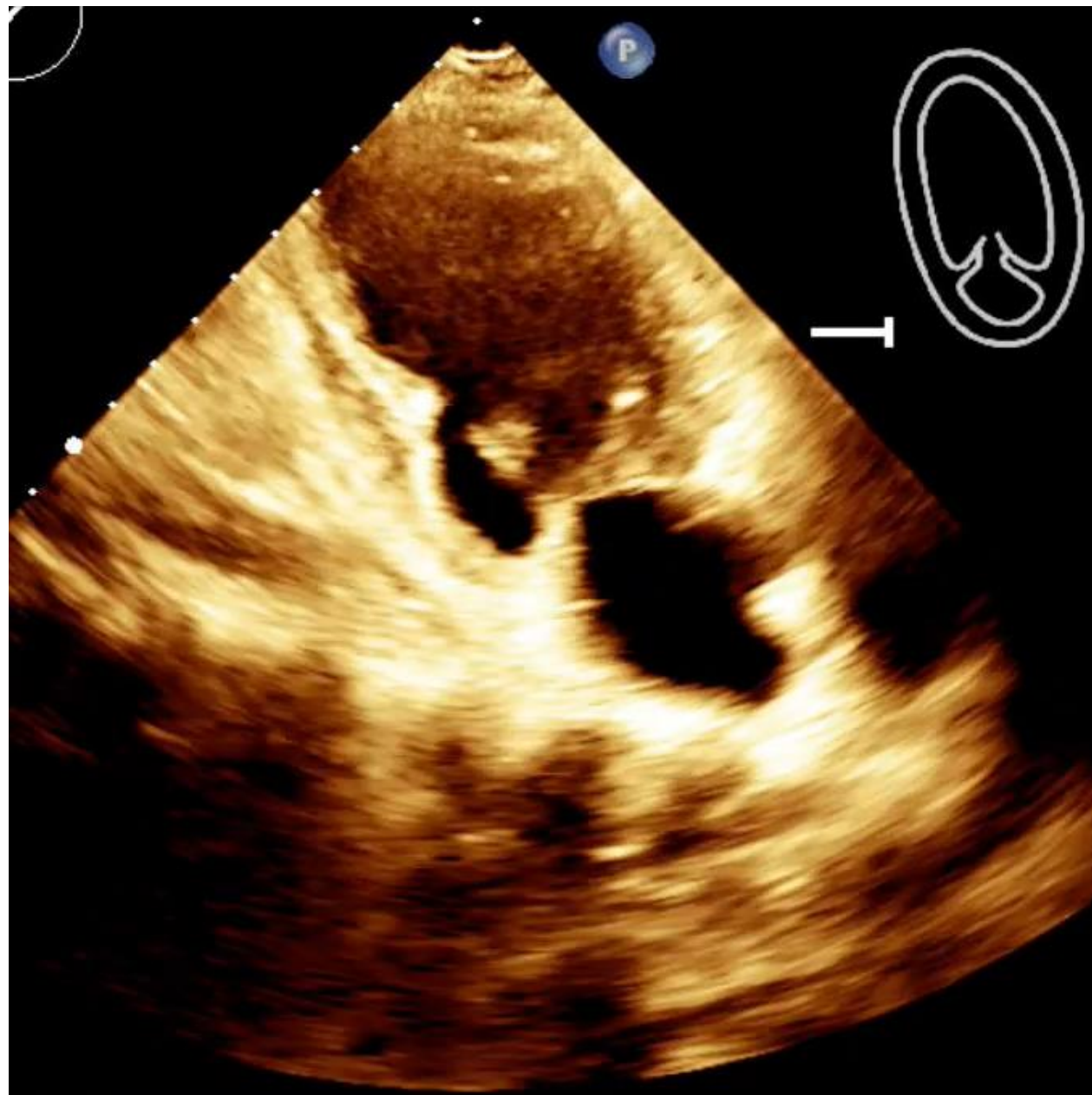
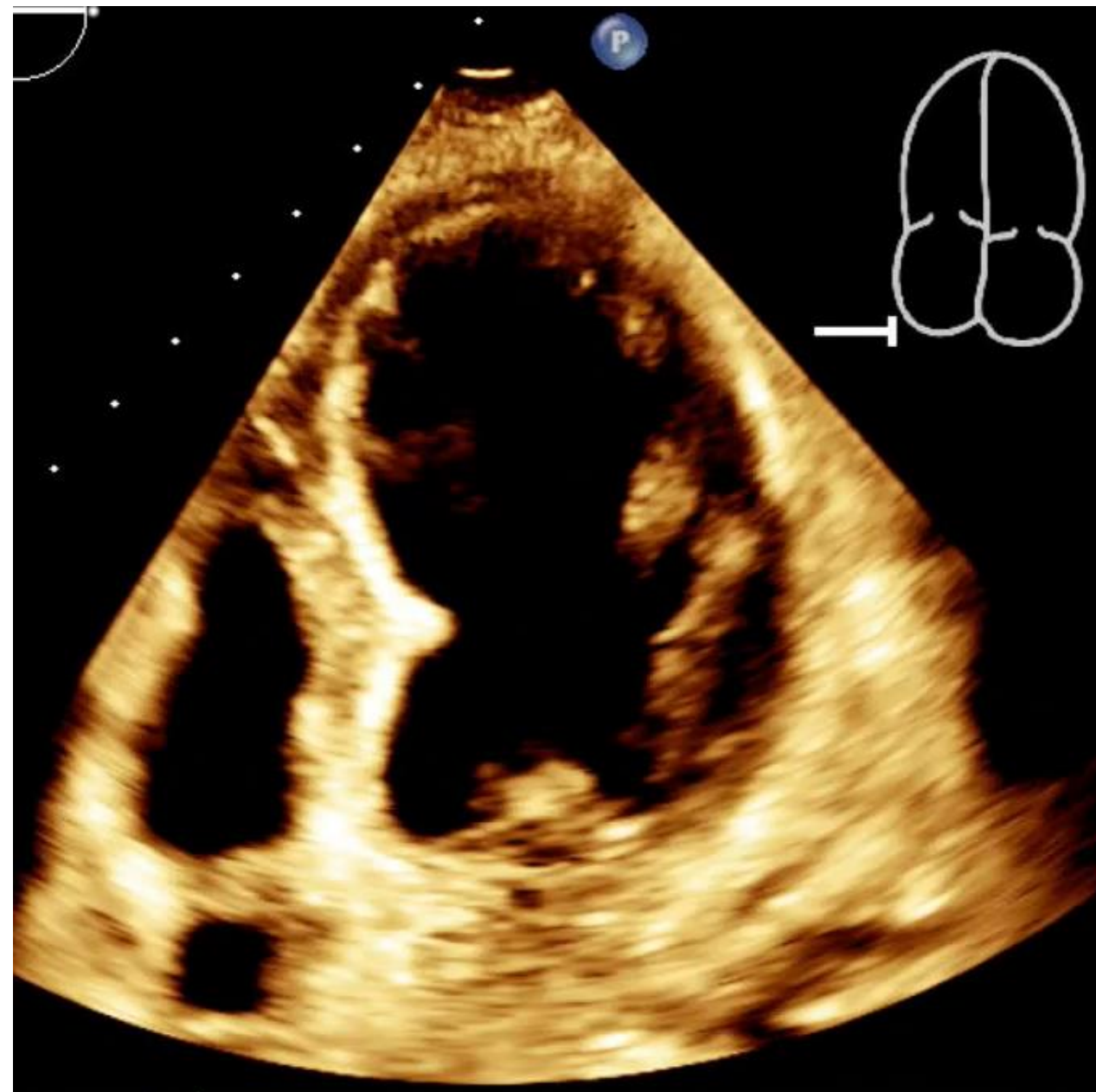


RA	20
RV	59/15
PA	66/44, mean 56
PCW	41
Fick	2.6 (1.7)
TD	2.4 (1.6)
PVR	6
LVEDP	36
Sys BP	96/63 (74)
CPO	0.41
PAPi	1.1

- Significantly elevated bilateral filling pressures
- Low cardiac output
- Elevated LVEDP
- In setting of severe triple vessel disease



- Admitted to CICU for AMI-CS
- Recommended inotropic support
- Urgent CABG consult



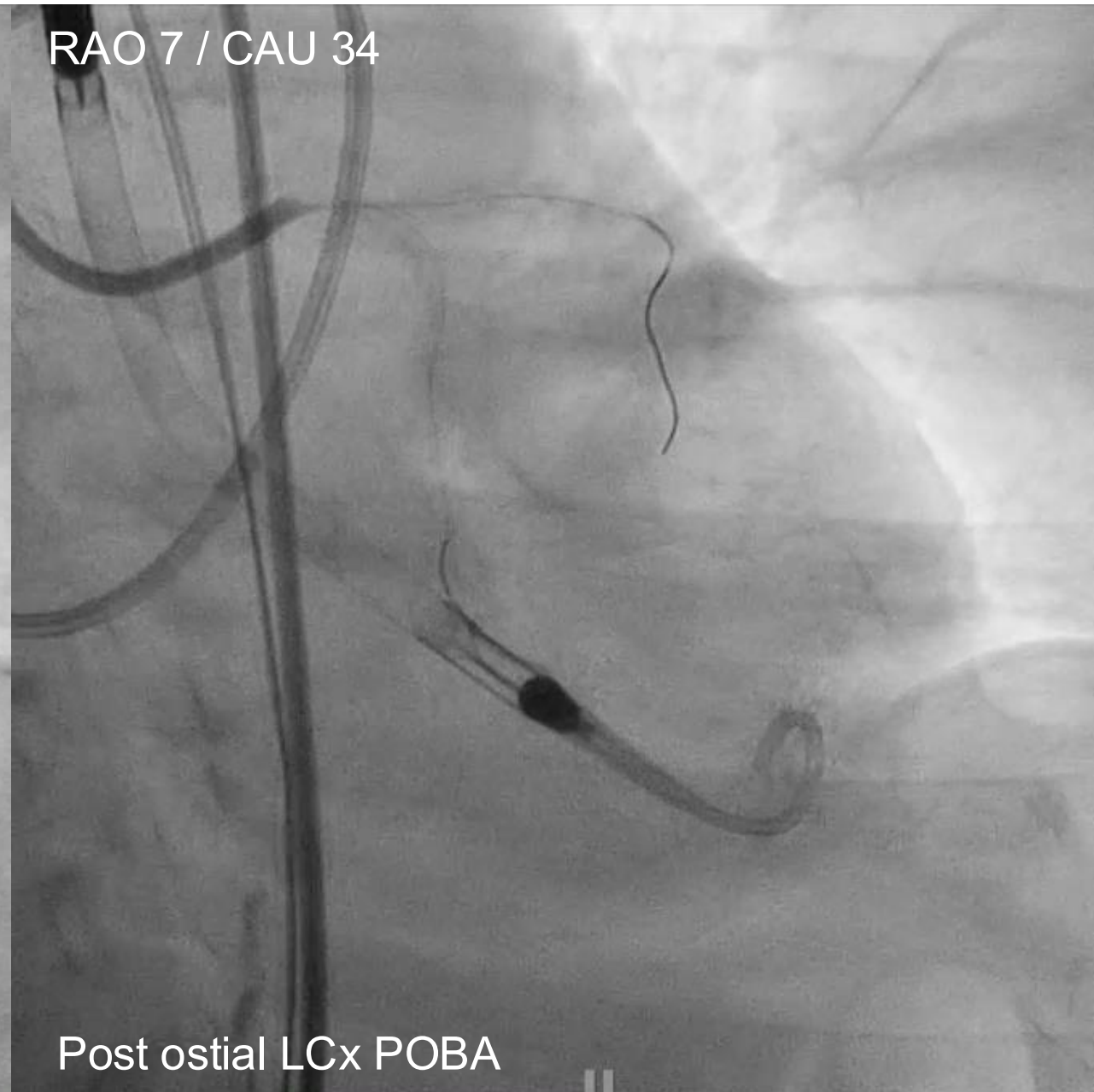
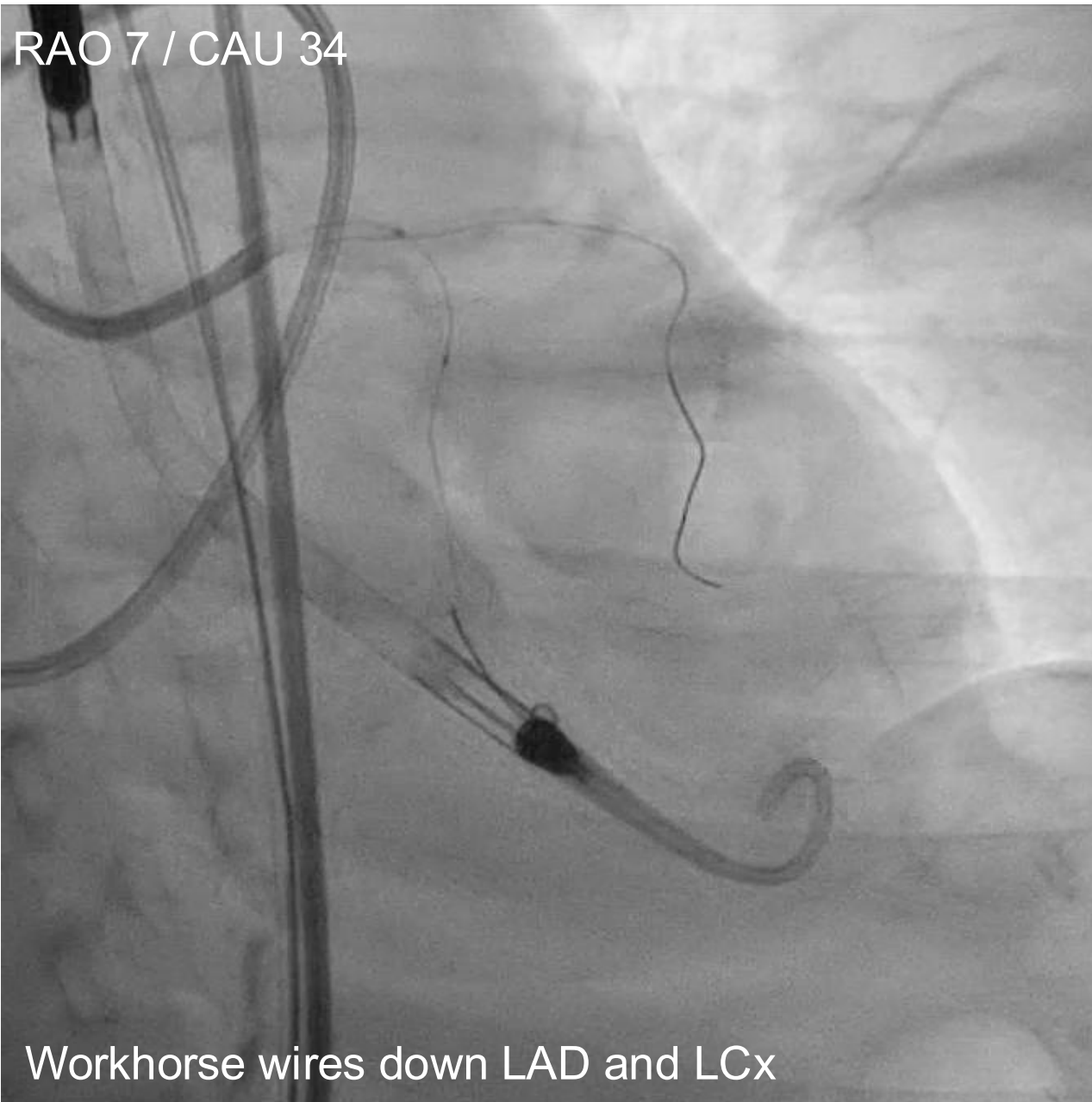
What would you do next?

- Severe 3V disease
 - LAD w/ possibly completed infarct based on TTE & ECG
 - Severe prox LCx disease
 - RCA CTO
- AMI-CS, SCAI C
 - Well supported on IABP 1:1
- ICM w/ HFrEF (20-25%)

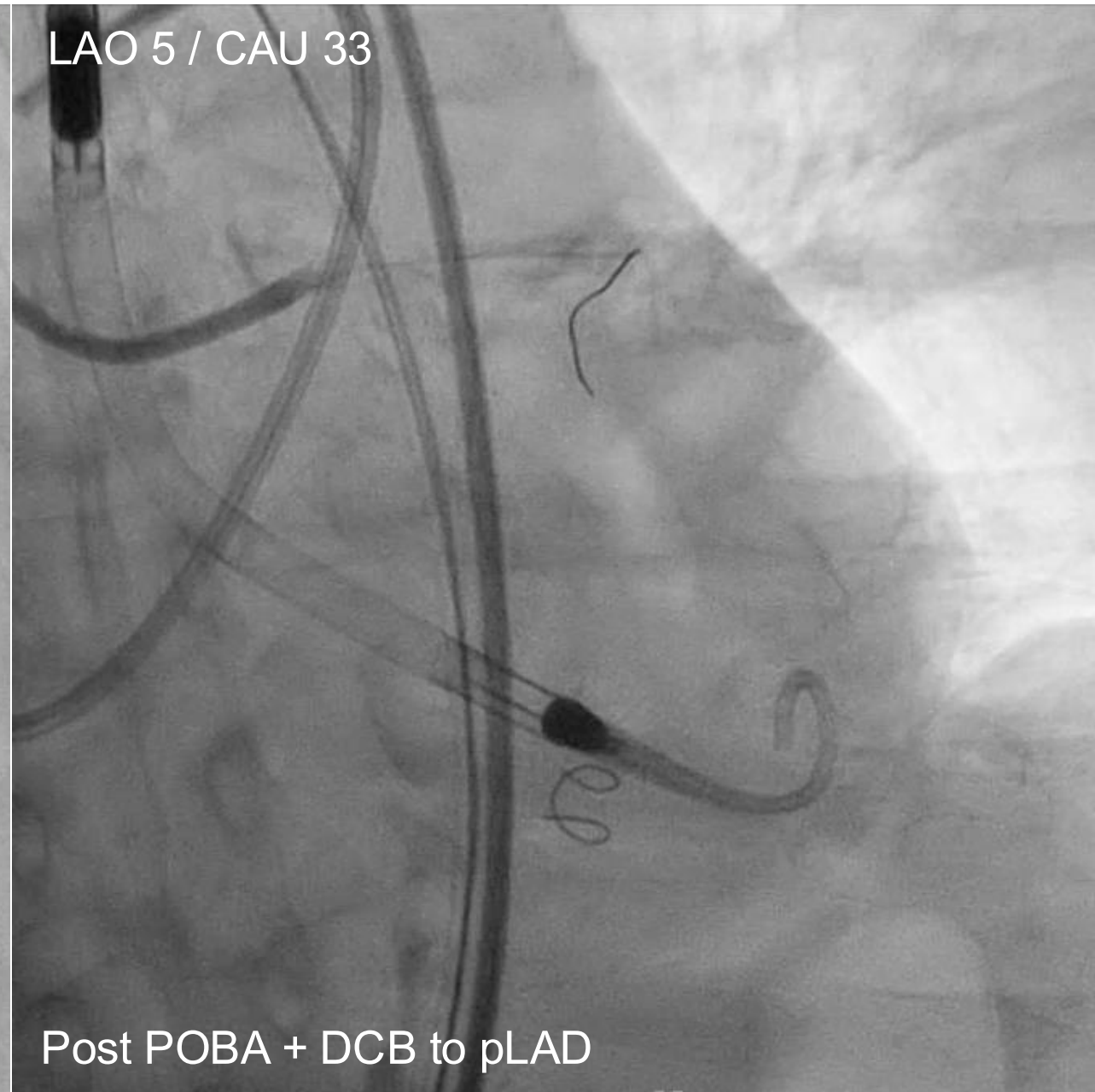
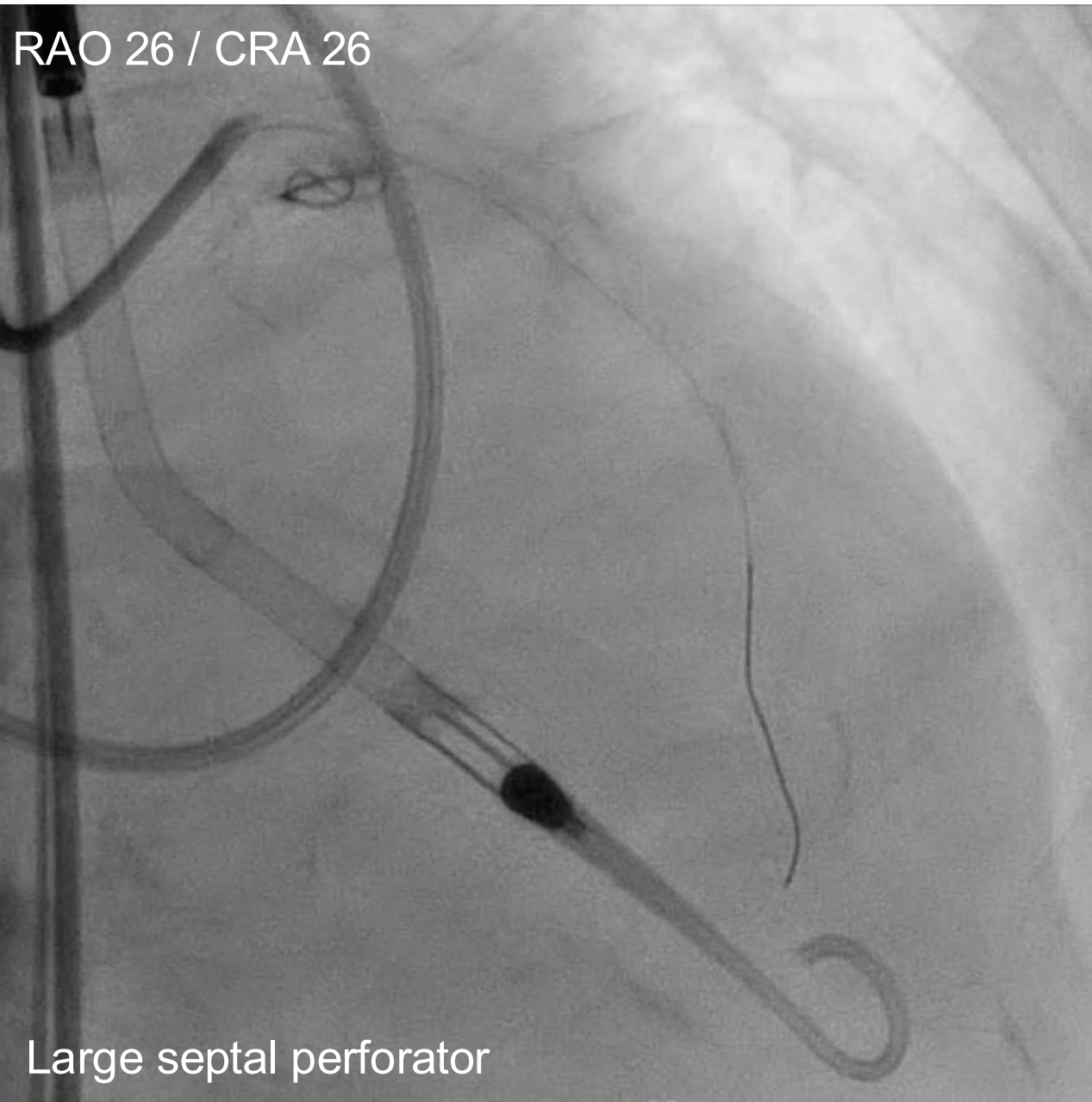
- ~~1. CABG~~
(strong patient preference)
2. High risk PCI
(revascularization targets / strategy?)
3. Continue or escalate tMCS as bridge to decision for VAD/OHT evaluation
(see 1^)
4. Medical management
5. Refer to palliative/hospice care

- Planned IABP exchange for periprocedural Impella CP support
- Upfront femoral central venous access
- Prioritizing targets:
 1. LM-LCx PCI/DES (“last remaining vessel”)
 2. RCA CTO
- Elective intubation in CICU prior to procedure
- Aorto-iliac duplex US vascular screening
- Pre-closure to be utilized for large bore access
- Cr stable at 0.7 to 1.0
- Shared decision making regarding very high-risk nature of procedure
 - quoted as >20% risk of MACE or other major adverse event

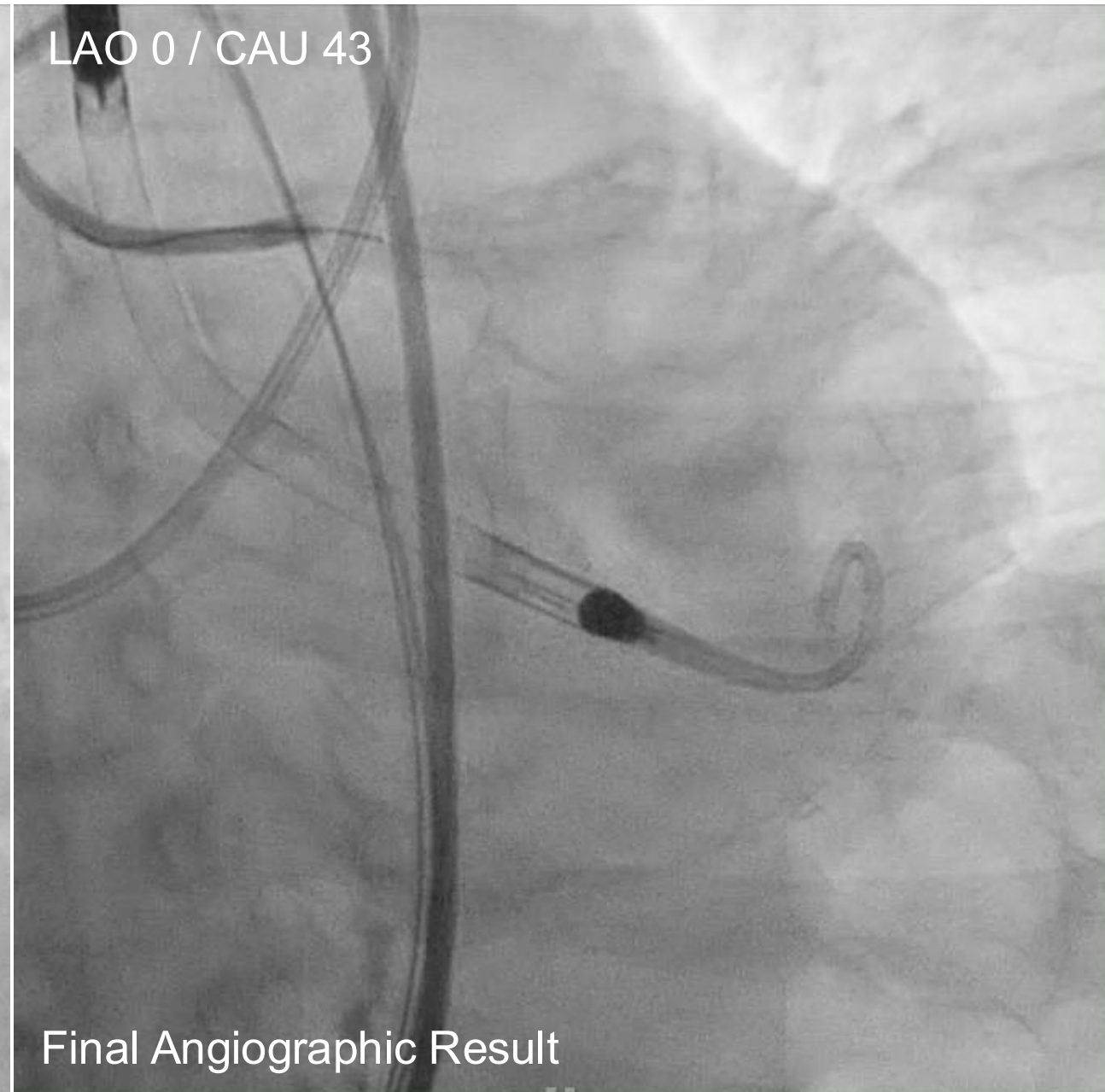
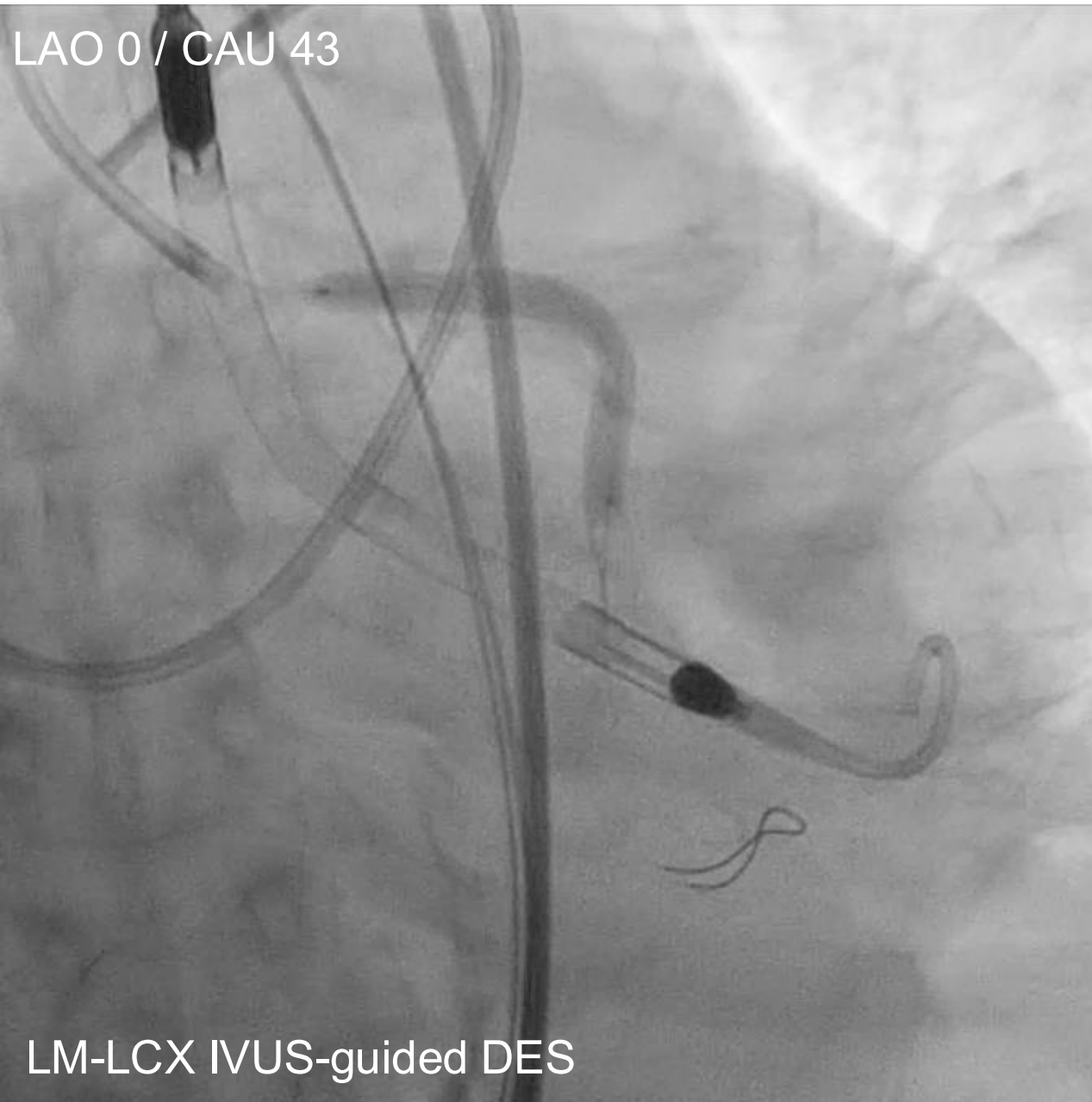
CHIP Procedural Images – L system



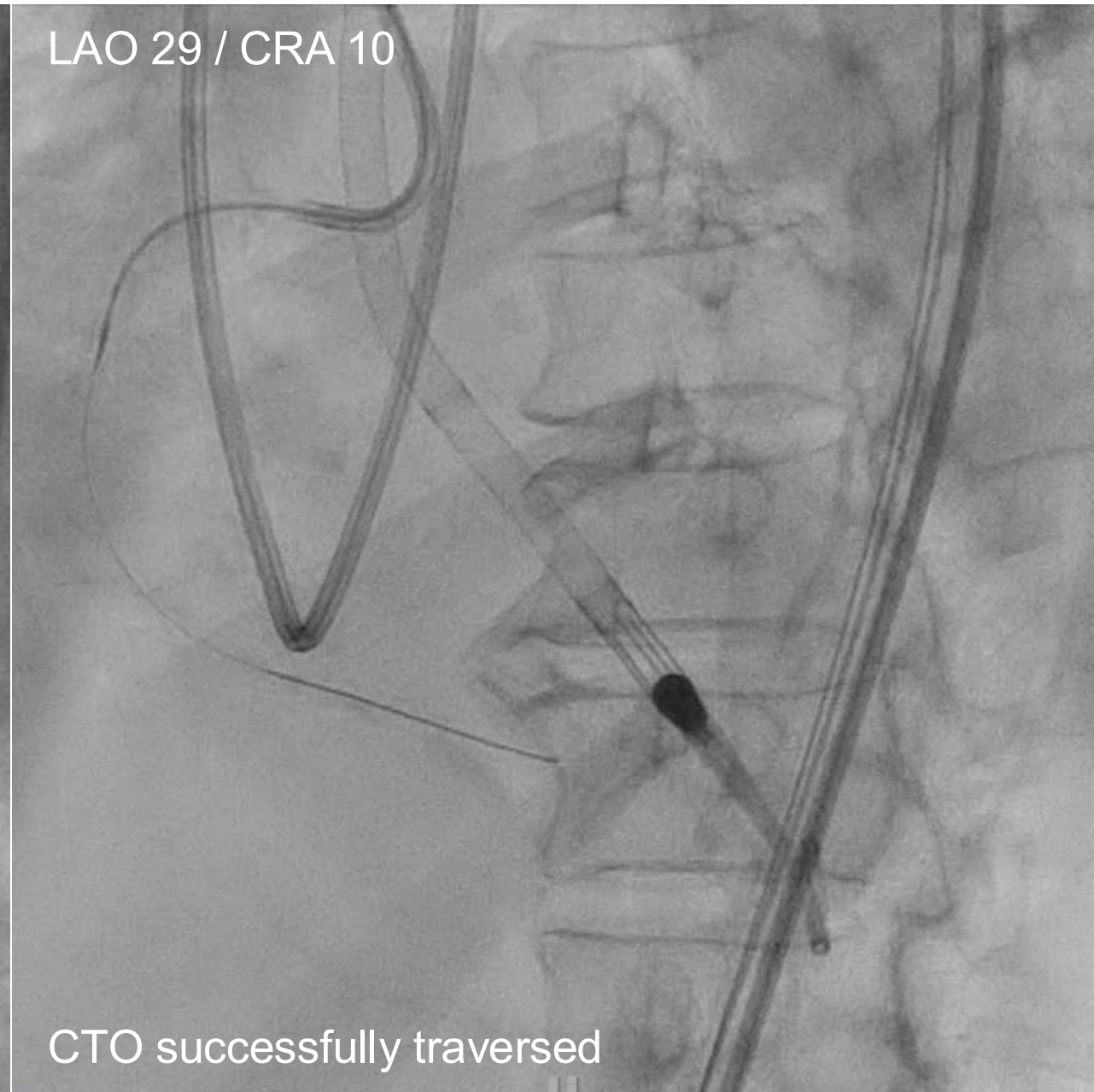
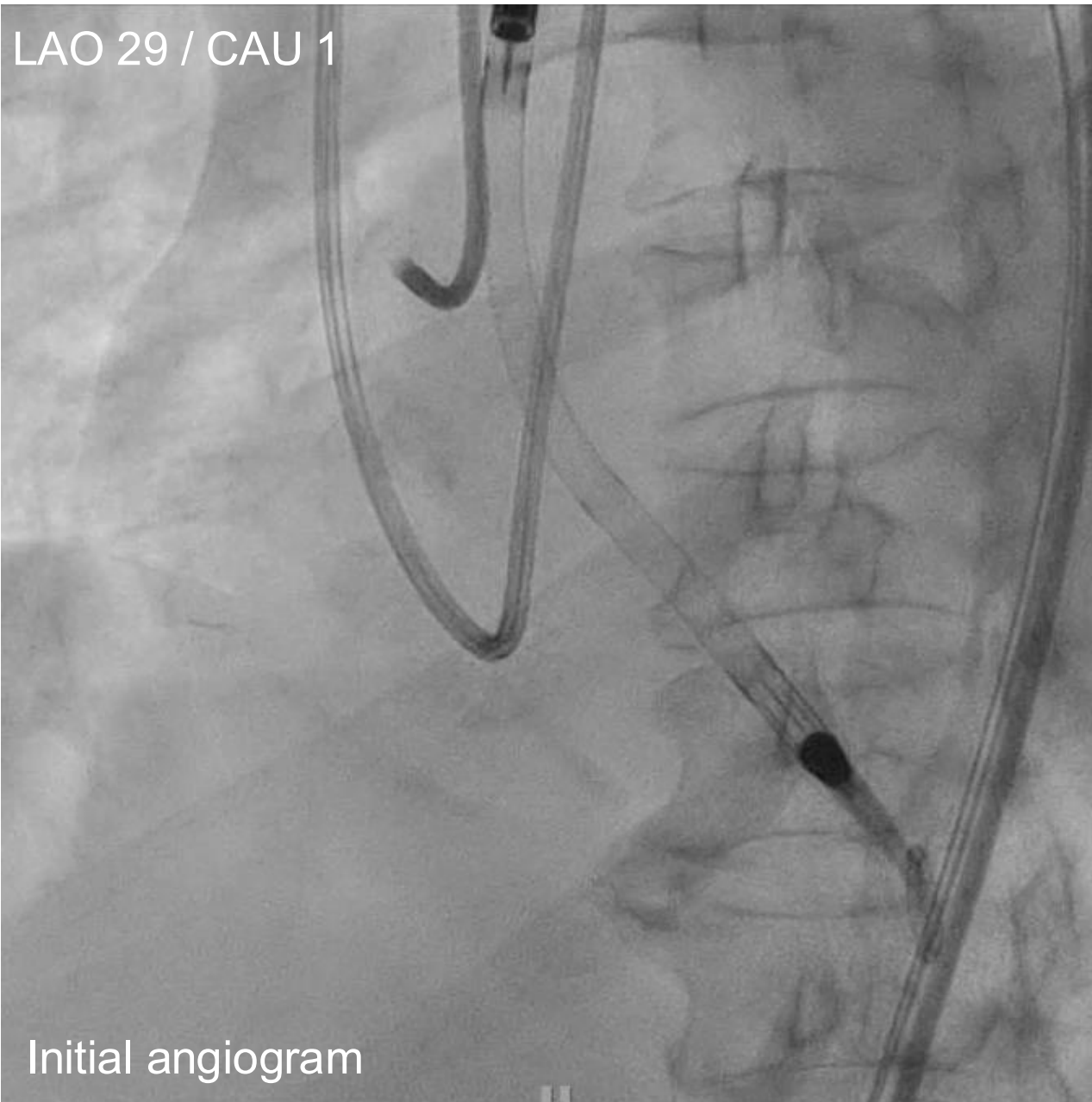
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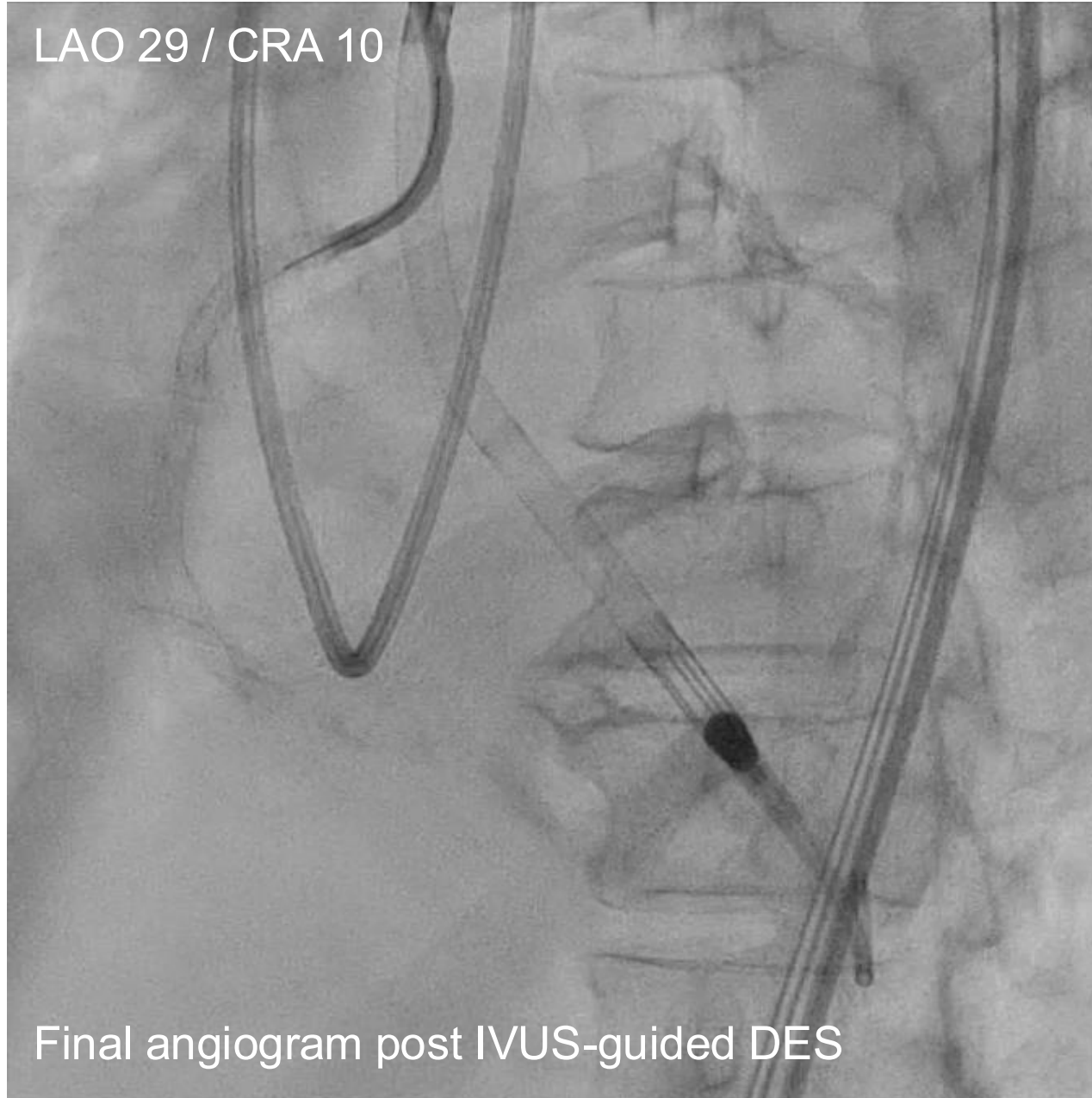
CHIP Procedural Images – L system



CHIP Procedural Images – RCA CTO



LAO 29 / CRA 10



Final angiogram post IVUS-guided DES

- Impella CP removed at end of case
- Returned to CICU intubated; extubated following morning
- Downgraded to floor POD 2
- Repeat TTE with EF recovery (20-25% -> 30-35%), similar WMA
- Discharged home POD 4
 - full GDMT regimen w/ DAPT (Brilinta + ASA) and Lipitor 80 mg
- Next steps:
 - cardiac rehab, GDMT optimization, surveillance TTE
 - consider viability study for staged LAD revascularization

Discussion

- Great example of the efficacy of Heart Team based care
 - Appropriate care escalation as patient was transitioning from SCAI B -> C
 - Reinforces importance of early engagement and frequent reassessment
- Challenging scenario of BiV AMI-CS due to “double jeopardy” / “wrap-around” MI
 - Requires high upfront clinical suspicion when ischemic ECG changes are present across multiple territories

Thank you

